

1. Administrative & Study Identification

Full Study Title: A Dose Escalation Study of Glofitamab (R07082859) as a Single Agent and in Combination With Obinutuzumab, Administered After a Fixed, Single Pre-treatment Dose of Obinutuzumab in Participants With Relapsed/Refractory B-cell Non-hodgkin's Lymphoma **Short Title/Acronym:** NP30179 Study **Protocol Number:** NP30179 (Also: 2016-001185-28, 2023-505625-14-00) **NCT Number:** NCT03075696 **Trial Status:** RECRUITING **Sponsor Name:** Hoffmann-La Roche (Genentech, Inc.) **Investigational Product:** Glofitamab (R07082859 / Columvi) and Obinutuzumab (Gazyva/Gazyvaro) **Phase of Development:** Phase I/II **Medical Monitor:** Hoffmann-La Roche Clinical Trial Medical Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, efficacy, tolerability, and pharmacokinetics (PK) of escalating doses of glofitamab, administered as a single agent and in combination with obinutuzumab, and to determine the Maximum Tolerated Dose (MTD) / Optimal Biological Dose (OBD) and Objective Response Rate (ORR). **Secondary Objectives:** To determine Complete Response (CR) rate, Duration of Response (DOR), Progression-Free Survival (PFS), Overall Survival (OS), Time to Response, and specific pharmacokinetic measurements.

2.2 Background & Rationale Patients with Relapsed/Refractory (R/R) B-cell Non-Hodgkin Lymphoma (NHL), including Diffuse Large B-Cell Lymphoma (DLBCL) and Mantle Cell Lymphoma (MCL), have a very poor prognosis following the failure of two or more lines of systemic therapy. There is a critical unmet need for novel therapies. Glofitamab is a bispecific CD20-directed CD3 T-cell engager designed to target CD20 on the surface of B-cells and CD3 on T-cells, redirecting T-cells to eliminate malignant B-cells. This study aims to establish safety, optimize step-up dosing to mitigate Cytokine Release Syndrome (CRS), and assess anti-tumor efficacy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose-escalation and Dose-expansion cohorts) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$\$\$: 920 participants **Number of Sites:** Multicenter (USA, Canada, 8 EU countries, etc.)

Estimated Study Start: Early 2017 **Estimated Primary Completion:** August 2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically-confirmed hematological malignancy expected to express CD20 (e.g., R/R B-cell NHL, DLBCL). 3. Relapse after or failure to respond to at least one (or two for expansion cohorts) prior treatment regimen. 4. Measurable disease, defined as at least one bi-dimensionally measurable nodal lesion > 1.5 cm or extranodal lesion > 1.0 cm. 5. No available treatment options expected to prolong survival (e.g., standard chemotherapy or autologous stem cell transplant). 6. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 7. Life expectancy of ≥ 12 weeks. 8. Adequate liver, hematological, and renal function. 9. Negative serologic or polymerase chain reaction (PCR) test results for acute or chronic Hepatitis B virus (HBV), Hepatitis C virus (HCV), and human immunodeficiency virus (HIV). 10. AEs from prior anti-cancer therapy must have resolved to Grade ≤ 1 .

4.2 Exclusion Criteria 1. Inability to comply with protocol mandated hospitalizations and restrictions (vital for CRS monitoring). 2. Participants with chronic lymphocytic leukemia (CLL), Burkitt lymphoma, and lymphoplasmacytic lymphoma. 3. Known or suspected history of hemophagocytic lymphohistiocytosis (HLH). 4. Active or previous central nervous system (CNS) lymphoma or disease, or history of confirmed progressive multifocal leukoencephalopathy (PML). 5. Acute bacterial, viral, or fungal infection at baseline (confirmed by positive blood culture within 72 hours prior to obinutuzumab). 6. Prior allogeneic stem cell transplantation (SCT).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Pre-treatment:** Obinutuzumab 1000 mg single dose IV infusion on Cycle 1 Day -7 (or Day 1) to deplete circulating/lymphoid B-cells. * **Glofitamab Step-Up Dosing:** 2.5 mg on Cycle 1 Day 8, 10 mg on Cycle 1 Day 15, and 30 mg on Day 1 of each subsequent 21-day cycle. **Route of Administration:** Intravenous (IV) Infusion. **Duration of Treatment:** Fixed duration of a maximum of 12 cycles (each cycle is 21 days).

5.2 Concomitant & Prohibited Therapy Permitted: Corticosteroids (e.g., dexamethasone) for premedication to prevent/reduce severity of CRS, and tocilizumab (Trade Name: Actemra, ATC Code: L04AC07) for the active management of CRS. **Prohibited:** Concurrent experimental anti-cancer therapies; live virus vaccines.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Days -28 to 0 | Informed Consent, Medical History, Tumor Assessment, Viral PCR/Serology | | **Cycle 1 (Pre-treatment)** | Day -7 / Day 1 | Obinutuzumab 1000mg IV infusion, safety labs | | **Cycle 1 (Step-Up)** | Days 8 & 15 | Glofitamab step-up doses (2.5mg and 10mg). **Mandatory 24-hour hospitalization** for CRS observation. | | **Interim** | Cycles 2-12 (Day 1) | Glofitamab 30mg IV, safety labs, efficacy/tumor assessment (Lugano criteria), AE monitoring | | **End of Study** | Post-Cycle 12 | Final restaging, follow-up for duration of response and overall survival |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * **Part I and II:** Percentage of Participants With Dose Limiting Toxicities (DLTs). * **Part I, II and III:** Percentage of Participants With Adverse Events (AEs). * **Part II:** Maximum Tolerated Dose (MTD) or Optimal Biological Dose (OBD) of Glofitamab.

7.2 Secondary & Exploratory Endpoints * **Pharmacokinetics (Part I, II and III):** Cmax of Obinutuzumab; Cmin of Obinutuzumab. * **Immunogenicity (Part I, II and III):** Anti-drug Antibodies (ADA) to Glofitamab. * **Efficacy:** Duration of Response (DOR), Complete Response (CR) rate, Progression-Free Survival (PFS), Overall Survival (OS), and Time to Response (TTR).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring for Cytokine Release Syndrome (CRS), Immune Effector Cell-Associated Neurotoxicity (ICANS), tumor flare, and serious infections.

Serious Adverse Events (SAEs): Boxed Warning protocols for severe/fatal CRS. Hospitalization is mandatory during and for 24 hours following the first and second step-up doses of glofitamab.

Data Monitoring Committee (DMC): Internal Safety Review Committee evaluating dose-escalation progression and cumulative toxicity.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Efficacy-evaluable population includes patients who received at least one study dose and had measurable disease at baseline. Safety Set includes all patients who received any amount of the investigational product. **Sample Size**

Justification: Expansion cohorts populated based on observed effect size to generate statistically significant bounds for ORR and DOR. **Handling of Missing Data:** Kaplan-Meier methodology used

for time-to-event endpoints (DOR, PFS, OS); patients without an event or lost to follow-up are censored at the date of last adequate tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: High-frequency onsite monitoring during initial cycles to ensure adherence to step-up dosing and CRS management protocols. **Audit Trail:** Standard oncology eCRF tracking with independent centralized radiological review for objective endpoints.

11. Study Identifiers & Governance

Primary ID: {{NP30179}} **Posting ID:** {{38640533695726238}}
Registry Number: {{NCT03075696}} **Sponsor/Lead Organization:** {{Hoffmann-La Roche}} **Study Title:** {{NP30179 Study - Glofitamab in R/R NHL}} **Official Regulatory Title:** {{A Dose Escalation Study of Glofitamab (R07082859) as a Single Agent and in Combination With Obinutuzumab, Administered After a Fixed, Single Pre-treatment Dose of Obinutuzumab in Participants With Relapsed/Refractory B-cell Non-hodgkin's Lymphoma}}

12. Clinical Framework (Lymphoma Specific)

Primary Condition: {{Relapsed/Refractory B-cell Non-Hodgkin Lymphoma (DLBCL, MCL)}} **Disease Area:** {{Non-Hodgkin's Lymphoma}} **Preferred UMLS Name:** {{Lymphoma, B-Cell, Non-Hodgkin}} **Semantic Type:** {{Neoplastic Process}} **MeSH Code:** {{D008228}} **HPO Code:** {{HP:0012539}} **SNOMED ID:** {{1172592001}} **Disease Definition:** {{Distinct from Hodgkin lymphoma both morphologically and biologically, non-Hodgkin lymphoma (NHL) is characterized by the absence of Reed-Sternberg cells, can occur at any age, and usually presents as a localized or generalized lymphadenopathy associated with fever and weight loss. The clinical course varies according to the morphologic type. NHL is clinically classified as indolent, aggressive, or having a variable clinical course. NHL can be of B-or T-/NK-cell lineage.}} **Diagnosis Threshold:** {{Histologically confirmed CD20+ NHL, Relapsed/Refractory to \$\\ge 1-2\$ prior lines}} **Medical Methodology:** {{T-cell engaging bispecific antibody therapy}} **Optimization Target:** {{Objective Response Rate (ORR) and Maximum Tolerated Dose (MTD)}}

13. Study Procedures & Methodology

Management Pathway: {{Obinutuzumab pre-treatment followed by Glofitamab step-up dosing}} **Education Protocol (HCP):** {{CRS and ICANS recognition, tox-management criteria, mandatory hospitalization rules}} **Education Protocol (Patient):** {{Neurological and CRS symptom awareness, infection risk guidelines}} **Quality Audit Mechanism:** {{Independent Review Committee (IRC) for tumor assessment via Lugano 2014 Criteria}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 12 Cycles (36 weeks) + Survival Follow-up}} **Onsite Visit Cadence:** {{High frequency Cycle 1 (Days -7, 1, 8, 15), then Day 1 of Cycles 2-12}} **Remote Monitoring Frequency:** {{Post-treatment follow-up for survival}} **Checklist Review Interval:** {{Every cycle for disease assessment and safety labs}}

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				